

PATIENT FIRST

2600 PAPERMILL ROAD
WYOMISSING PA 196103362
484/220-0051

PAUL RIVERA JR-WC

10.17.22 09:50

BMI-High

Your BMI today was 34.3.

Body Mass Index (BMI) is a number calculated from a person's height and weight. The Centers for Disease Control has determined that BMI is an indicator of potentially unhealthy weight for many people. The target BMI for persons ages 18 and older is between 18.5 and 24.9.

Your BMI may differ from the above standards due to your level of fitness or other factors; for example, athletes may be outside the target BMI range for their age.

You are receiving these instructions because your BMI today is above the target range for your age. Patient First is providing these instructions to our patients consistent with good medical practice and with requirements of federal government and commercial insurance health management programs.

Being above your target BMI range increases your risk of heart disease, diabetes, and some types of cancer. You may be able to reduce these risks and reach your target BMI by losing weight. The keys to losing weight and keeping it off are a healthy diet, regular physical activity, and eating fewer calories than your body uses until you reach your target BMI. Your initial weight-loss goal should be 10% of your body weight over the next 6 months. You should not expect to lose more than 1 or 2 pounds a week.

Here are some suggestions to get you started:

1. Change your diet: Eat 500 fewer calories a day. This can lead to weight loss of one pound per week. Make healthy food choices such as fruits and vegetables, salads, fish, lean meat, and whole wheat bread. Decrease portion sizes. Avoid soft drinks and minimize alcohol intake.
2. Increase physical activity: Exercising helps burn extra calories. It also strengthens your heart, decreases your risk of developing many medical problems, and may help you lose weight and keep it off over time. Walking 30 minutes per day, 5 days a week is a good start. If you develop shortness of breath, lightheadedness, or chest pain while exercising, stop exercising and see your physician immediately.

Return to Patient First or to your primary care physician for follow-up in 1 month, 3 months, and 6 months. For more information and tips, visit <http://www.cdc.gov/healthyweight/assessing/bmi/index.html>.

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Smoking/Tobacco Use

Smoking is dangerous to your health and increases your risk of developing high blood pressure, stroke, heart attack, cancer, decreased lung function and reduced life expectancy. Smoking also increases the production of stomach acid and is contraindicated in patients with peptic ulcer disease or esophageal reflux. Discontinuing a smoking habit can be difficult and requires significant "will power." Nicotine patches or Nicorette chewing gum can be helpful aids to you as you try to stop smoking. If you receive a prescription for patches or gum, you should stop smoking and remove all cigarettes from home, car, and work place.

Smokeless tobacco includes chewing tobacco, dry and moist snuff and snus. All of these contain nicotine. These products have not been proven to help smokers quit smoking. Harmful effects from smokeless tobacco include mouth, tongue, cheek, gum, and throat cancer; cancer of the esophagus, stomach, and pancreas; possible increase in risk of heart disease, heart attacks, and stroke; mouth sores that may become cancer, gum disease, tooth decay, and tooth loss.

If you need help in stopping smoking or other tobacco use, call toll free 1-800-QUIT-NOW (1-800-784-8689) or 1-855-DEJLO-YA (1-855-3569) for phone-based counseling. There is no charge for this service.

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BP, Adult

Your lowest systolic and lowest diastolic blood pressures today were: 148/86.

High Blood Pressure (Hypertension) is a common medical problem which may affect you without your being aware of it. Untreated high blood pressure is associated with increased risk of stroke, heart attack and decreased life span. Your blood pressure today was elevated. One elevated blood pressure reading does not necessarily mean that you have a problem. It may take several readings to make that determination.

A normal blood pressure (BP) reading for an adult is less than 120 systolic (top number) and less than 80 diastolic (bottom number). A BP with the top number 120-139 or the bottom number 80-89 is considered "prehypertension." A BP with the top number 140-159 or the bottom number 90-99 is Stage 1 hypertension. A BP with the top number 160 or higher or the bottom number 100 or higher is Stage 2 hypertension.

Lifestyle modifications recommended for prevention and treatment of high BP:

- 1) Lose weight if overweight.
- 2) Limit daily alcohol intake (24 oz beer, 10 oz wine, or 2 oz 100-proof whiskey for adult male and half that amount for adult female or lighter weight male.)
- 3) Increase aerobic physical activity (30-45 minutes most days).
- 4) Reduce sodium intake to no more than 2,400 mg sodium per day. Do not add salt to food at the table and avoid canned soups and vegetables.
- 5) Maintain adequate intake of dietary potassium preferably from fresh fruits and vegetables.
- 6) Maintain adequate intake of dietary calcium and magnesium for general health.
- 7) Stop smoking and reduce intake of dietary saturated fat and cholesterol for overall cardiovascular health.

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X-ray Readings

You had one or more x-ray studies taken today at Patient First. Your Patient First physician or physician extender has given a primary reading of the x-ray study/studies and has made treatment recommendations based on this primary reading. This reading will be reviewed tomorrow by a radiologist, whose review usually will be posted on your chart by 7 p.m. tomorrow evening.

If there is a significant difference between the radiologist's reading and the primary reading, we will attempt to notify you by phone. There are two situations that deserve special mention.

If we have advised you that you had a fracture, but the radiologist did not see a fracture, we will often suggest you follow-up with an orthopedic surgeon or return to Patient First as originally advised. This is because the Patient First physician or physician extender made the diagnosis based on both the x-ray and his/her evaluation of your condition, whereas the radiologist only saw the x-ray. When this is the case, it is best to continue to treat for a fracture until another visit to a physician, at which time the original diagnosis may be confirmed or refuted.

In some cases, an x-ray may not support what is considered to be a "clinical" diagnosis made based on your signs and symptoms at the time of your visit. Pneumonia is an example of a "clinical" diagnosis that may not be supported by x-ray findings. One reason is that x-rays for pneumonia may not show the pneumonia until you have been sick for several days. Therefore, we may call you and advise that the radiologist did not see pneumonia on your x-ray, but you should continue to take your medications and follow-up as scheduled.

If you have any questions or concerns about your x-ray reading, you may call the Patient First center at which you were seen and speak with a nurse.

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